



JDScience BTEC Health and Social Care Unit 1 Set C mark scheme

Original JDScience mark scheme. Indicative content only. Equivalent wording that meets the command word should be credited. Not an official Pearson paper.

Use this booklet after the matching practice paper. Do not issue it with the student paper.

Time: 1 hour 30 minutes · **Total marks:** 90

Question 1 · 26 marks

Sophie is 16 and 28 weeks pregnant. She lives with her mum. She has stopped attending college three days a week because of nausea and tiredness. She wants to keep the baby. Her friends have been less in touch. The midwife has given her information about antenatal classes and college support.

(a) [2 marks]

Identify two physical developments that occur during pregnancy for the mother.

- Examples: uterine growth; weight gain; breast changes; increased blood volume; tiredness/nausea as physical effects.

(b) [4 marks]

Describe two intellectual effects that unexpected pregnancy may have on an adolescent.

- Interrupted college learning; difficulty concentrating because of tiredness.
- Need to learn about pregnancy, birth and infant care — new knowledge.

(c) [6 marks]

Explain how Sophie's emotional and social development may be affected.

- Emotional: anxiety, excitement, identity shift from student to parent-to-be; possible conflict at home.
- Social: friends less in touch → isolation; mum and midwife as support; antenatal classes as a new social group.

(d) [14 marks]

Discuss how nature and nurture both influence the unborn baby's development in this case.

- Nature: genetics from Sophie and the baby's father; gestation at 28 weeks — organs still maturing.
- Nurture: maternal nutrition, rest, antenatal care, stress, college attendance affecting Sophie's health which affects the foetus.
- Avoid blaming Sophie; keep it applied and balanced.
- Level 3: both sides with a conclusion that both matter.

Question 2 · 22 marks

Mr Idris is 64. He had a stroke three months ago. His right arm is weaker, his speech is slower and he feels low. He was a bus driver. His partner has reduced her hours to help him. He has started physiotherapy and a stroke group.

(a) [2 marks]

Identify two physical effects of a stroke.

- Weakness/paralysis on one side; speech difficulty (dysarthria/aphasia); fatigue; balance problems; swallowing difficulty.

(b) [4 marks]

Describe two intellectual or communication changes Mr Idris may experience.

- Slower speech; word-finding; possible attention/memory fatigue after the event.
- Need to relearn sequences (dressing, using a phone).

(c) [6 marks]

Explain how this life event may affect Mr Idris's emotional development.

- Loss of driving role → grief, frustration, low mood.
- Dependence on partner may affect self-esteem and the relationship.
- Stroke group and physiotherapy can restore hope and control.

(d) [10 marks]

Evaluate the support available to promote Mr Idris's development after the stroke.

- Physiotherapy: physical recovery and independence.
- Stroke group: social/emotional peer support.
- Partner: daily support but risk of carer strain — services should support both.
- Possible speech and language therapy, occupational therapy, benefits advice (apply, do not dump a list).
- Level 3: judgement about what will help most and any gaps.

Question 3 · 20 marks

The Holmes and Rahe Social Readjustment Rating Scale lists life events that may cause stress. Amira, 33, has in one year: got married, moved city for work, become pregnant and had a bereavement in her extended family. She is sleeping poorly.

(a) [2 marks]

Identify two examples of expected life events.

- Examples: starting school; marriage; starting work; retirement; moving house (can also be unexpected). Award expected events, not disasters.

(b) [4 marks]

Outline two unexpected life events.

- Bereavement; serious illness/accident; redundancy; relationship breakdown. Outline means a brief phrase, not a full essay.

(c) [6 marks]

Explain how a cluster of life events may affect Amira's physical and emotional development.

- Cumulative stress (Holmes and Rahe idea): sleep loss, immune/physical strain, anxiety.
- Positive events (marriage, pregnancy, new job) still require adjustment.
- Pregnancy plus poor sleep affects energy; bereavement may complicate joy about the baby.

(d) [8 marks]

Discuss ways Amira could be supported to reduce the negative effects of these events.

- Midwife/GP: sleep, mental health, antenatal care.
- Employer: reasonable adjustments, maternity information.
- Social: partner, friends, bereavement support.
- Self-care: rest, realistic expectations — not a substitute for services.
- Level 3: range of supports with a conclusion.

Question 4 · 22 marks

A nursery reports on two children aged 18 months. Hanna walks confidently, uses 20 words and waves goodbye. Yusuf has low muscle tone, sits with support and uses a few sounds. Yusuf has weekly physiotherapy. Both children have attentive parents.

(a) [2 marks]

Identify two gross motor skills expected by 18 months.

- Walking independently; crawling; pulling to stand; climbing onto a sofa with help; kicking a ball (emerging).

(b) [4 marks]

Describe two fine motor skills expected in infancy or early toddlerhood.

- Pincer grasp; pointing; stacking a few blocks; scribbling; self-feeding with fingers/spoon with mess.

(c) [6 marks]

Explain why children of the same age may show different patterns of development.

- Biological: low muscle tone, possible additional needs.
- Experience: opportunity to practise; physiotherapy as planned intervention.
- Development is a range, not a single timetable; delay needs monitoring but is not automatically neglect — both have attentive parents.

(d) [10 marks]

Assess how early intervention could affect Yusuf's later development.

- Physiotherapy now may improve tone, sitting, later walking and confidence.
- Language: sounds now — SALT if needed to support later communication and social play.
- Parent involvement: practising at home multiplies effect.
- Without support, delay might widen (catch-up vs widening gap).
- Level 3: reasoned assessment of likely benefit and remaining uncertainty.